

Policy on Informed Consent

«Hospital Name»

Document control

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Prepared by	Patient Rights Officer Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

«Hospital Name» obtains valid informed consent, from the patient or the person who may consent on their behalf, before any procedure on its written consent list — as a process of effective communication about the procedure, its risks, benefits, alternatives, and who will perform it, in a language the patient can understand.

1. Purpose

This policy explains how «Hospital Name» obtains informed consent from the patient or family about their care: maintaining a written list of procedures requiring consent, following statutory norms for the consent process, ensuring consent includes procedure information in an understandable language, describing who may consent when the patient cannot, and ensuring the person performing the procedure is responsible for the consent process.

The definition of specific invasive/high-risk procedures requiring consent under the hospital's rights policy is covered in the hospital's other policies.

2. Scope

This policy applies to all doctors and clinical teams performing procedures on the organisation's informed-consent list, and to all patients and persons who may consent on a patient's behalf.

3. Policy standards

«Hospital Name» maintains a written list of procedures requiring informed consent and written guidance on the consent process, taking into account applicable statutory requirements such as the MTP Act, PC-PNDT Act, Transplantation of Human Organs Act, and HIV and AIDS (Prevention and Control) Act 2017/NACO HIV-testing policy where relevant to the services provided.

Consent is taken before the procedure, witnessed, includes the procedure's risks, benefits, alternatives and who will perform it, is obtained from the patient or the person who may consent when the patient is incapable, and is the responsibility of the person performing the procedure.

4. Non-negotiable rules

- Do not start a procedure on the organisation's informed-consent list until valid informed consent has been obtained from the patient, or from the person who may consent when the patient is incapable of independent decision-making, in a language they can understand.
- Do not accept a consent form signed by a nurse or clerk as the only consent when the person performing the procedure (or a doctor member of that team) has not explained the procedure, its risks, benefits, alternatives and who will perform it.
- Do not allow anyone to consent on behalf of a competent adult.
- Do not use a consent validity period longer than six months for a procedure repeated over a long time without obtaining fresh consent.
- Do not change or add a treatment modality without obtaining fresh consent.

5. What we do

5.1 Maintain a written consent list and consent process guidance

A written list of procedures requiring informed consent is maintained, taking into account this standard's requirements and applicable statutory requirements. Written guidance explains the steps of the informed consent process and the person responsible. Staff are made aware of this guidance.

5.2 Follow statutory norms in the consent process

Consent is taken before the procedure. At least one witness — present for the entire duration of the doctor-patient communication — signs the consent form. For procedures repeated over a long time (for example, dialysis), consent is taken at the first instance with a defined validity period not exceeding six months, and the patient endorses the consent at each repeat treatment. Fresh consent is obtained if there is a change in treatment modality or an addition of another modality.

5.3 Ensure consent includes procedure information in an understandable language

Consent includes information regarding the procedure, its risks, benefits, alternatives, and who will perform it, in a language the patient can understand. The consent names the doctor performing the procedure; where multiple specialties are involved, it names the principal surgeon from each specialty, and each doctor explains their own role, risks, benefits and alternatives. If a doctor under training performs the procedure, this is specified along with the name of the supervising qualified doctor. The consent form is at a minimum bilingual, and where consent is taken in a language other than what the patient understands, the language of counselling and any interpreter used is clearly documented.

5.4 Describe who may consent when the patient is incapable

Consent is taken from the patient when the patient is capable and above the legal age for giving consent; no one may consent on behalf of a competent adult. When the patient is incapable of independent decision-making, «Hospital Name» follows applicable statutory norms and takes consent from the next of kin/legal guardian in the order: spouse, son/daughter, parents, brothers/sister. For life-threatening situations where the patient is incapable and next of kin is not available, the treating doctor and another clinician may decide to safeguard the patient's life; both names and the reason are documented.

5.5 Ensure the performer is responsible for consent

The person performing the procedure is responsible for the entire consent process, including explanation and taking the signature. It is not acceptable for the person performing to only explain while a nurse or clerk takes the written consent. A doctor member of the performing team may take consent on behalf of the person performing the procedure.

6. Stop-work authority

Do not start a procedure on the organisation's informed-consent list until valid informed consent has been obtained from the patient, or from the person who may consent when the patient is incapable of independent decision-making, in a language they can understand.

Do not accept a consent form signed by a nurse or clerk as the only consent when the person performing the procedure (or a doctor member of that team) has not explained the procedure, its risks, benefits, alternatives and who will perform it.

Stop-work applies to the procedure start. Immediate life-saving care when the patient is incapable and next of kin is not available follows the two-clinician emergency rule in Section 5.4, and is documented the same shift.

The person who stops tells the person performing the procedure and the Medical Superintendent the same shift. Refusing to start without valid consent is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for ensuring the informed-consent list and process are maintained and followed; receives stop-work escalations.
Patient Rights Officer	Owns implementation of this policy; maintains the consent list and process guidance; audits consent forms.
Person Performing the Procedure	Responsible for the entire consent process — explanation and signature — for their own procedure; receives stop-work escalations for their procedure.
Nursing Superintendent	Ensures nursing staff do not accept consent forms without the performer's explanation; escalates stop-work triggers.
Quality Coordinator	Audits this policy; holds training records, staff acknowledgements, and stop-work event logs.

8. Quality monitoring

Monitoring area	What is monitored
Consent list and guidance	Written consent list current; process guidance available; staff awareness records.
Statutory norms	Witness signatures; validity periods for repeat procedures; fresh-consent records on modality change.
Consent content and language	Bilingual consent forms; documentation of performer name(s), risks, benefits, alternatives; interpreter records where used.
Consent for incapable patients	Next-of-kin consent records showing order of preference followed; two-clinician emergency decision records.
Performer responsibility	Consent forms showing the performer or team doctor as the responsible party; stop-work event log.

9. Training and staff acknowledgement

All doctors and clinical teams who perform procedures on the informed-consent list shall be familiar with the consent process, statutory norms, and stop-work triggers in this policy, at induction and periodically.

I have read the Policy on Informed Consent of «Hospital Name» and understand my responsibilities under it.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to all doctors and clinical teams, the Patient Rights Officer, department heads, and the Quality Coordinator.

11. Abbreviations

Abbreviation	Meaning
CAPA	Corrective and Preventive Action
MTP	Medical Termination of Pregnancy
NABH	National Accreditation Board for Hospitals and Healthcare Providers
NACO	National AIDS Control Organisation
PC-PNDT	Pre-Conception and Pre-Natal Diagnostic Techniques

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means the objective element is starred and requires documented evidence.

Objective Element	Level	Traceability to this policy
PRE.4.a	CORE*	Sections 3, 5.1 and 6 address the written consent list, process guidance, staff awareness, and the stop-work trigger for procedures started without consent.
PRE.4.b	Commitment	Sections 3 and 5.2 address statutory norms in the consent process, including witness and repeat-procedure validity requirements.
PRE.4.c	CORE	Sections 3, 5.3 and 6 address consent content, language, multi-specialty consent, and the stop-work trigger for consent taken without performer explanation.
PRE.4.d	Commitment*	Sections 3 and 5.4 address who may consent when the

		patient is incapable, including the order of preference and the two-clinician emergency rule.
PRE.4.e	CORE	Sections 3, 5.5 and 6 address performer responsibility for the consent process and the related stop-work trigger.

13. Required Records/Evidence Checklist

Consent list and process guidance — PRE.4.a (CORE*)

- Written list of procedures requiring informed consent.
- Written consent process guidance naming the person responsible.
- Staff awareness records.
- Completed consent forms for listed procedures.

Statutory norms in consent — PRE.4.b (Commitment)

- Consent forms showing pre-procedure signature and witness signature.
- Validity period documentation for repeat procedures (not exceeding six months).
- Patient endorsement records at repeat treatments.
- Fresh-consent records on treatment modality change.

Consent content and language — PRE.4.c (CORE)

- Completed bilingual consent forms naming the performer(s), risks, benefits and alternatives.
- Interpreter records where used.
- Multi-specialty consent forms where applicable.

Consent for incapable patients — PRE.4.d (Commitment*)

- Written description of who may consent when the patient is incapable, including order of preference.
- Consent forms showing next-of-kin signatory and relationship.
- Two-clinician emergency decision records, when applicable.

Performer responsibility — PRE.4.e (CORE)

- Consent forms showing the performer or team doctor as the responsible party.
- Audit records of consent form review.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. PRE.4.
2. Medical Termination of Pregnancy Act; Pre-Conception and Pre-Natal Diagnostic Techniques Act, 1994; Transplantation of Human Organs Act; HIV and AIDS (Prevention and Control) Act, 2017 and the National AIDS Control Organisation's national HIV-testing policy — applicable to «Hospital Name» only to the extent its services fall within the scope of these statutes.

Disclaimer

This policy reorganises the supplied PRE.4 objective-element wording and Guidebook interpretation into plain-language guidance for «Hospital Name». It does not replace the NABH Accreditation Standards for Hospitals, 6th Edition, or the Guidebook, which remain the authoritative source in case of any conflict.

This policy references certain statutes (MTP Act, PC-PNDT Act, Transplantation of Human Organs Act, HIV and AIDS (Prevention and Control) Act 2017) as examples of statutory requirements that may shape «Hospital Name»'s consent list, where its services fall within their scope. This policy is not a statement of, or substitute for, compliance with any of these statutes; «Hospital Name» remains separately responsible for meeting all applicable statutory and regulatory requirements.

This policy shall be reviewed at least once every year, or earlier if NABH standards or applicable law changes.

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